

1. Administrative & Study Identification

Full Study Title: A PHASE 1 DOSE ESCALATION AND EXPANSION STUDY TO EVALUATE SAFETY, TOLERABILITY, PHARMACOKINETIC, PHARMACODYNAMIC, AND ANTITUMOR ACTIVITY OF PF-07820435 AS MONOTHERAPY AND IN COMBINATION IN PARTICIPANTS WITH ADVANCED SOLID TUMORS **Short Title/Acronym:** A Study of PF-07820435 as a Single Agent and in Combination in Participants With Advanced Solid Tumors **Protocol Number:** C5391001 **NCT Number:** NCT06285097 **Sponsor Name:** Pfizer **Investigational Product:** PF-07820435 (Oral STING agonist) and Sasanlimab (anti-PD-1 biologic) **Phase of Development:** Phase 1 **Medical Monitor:** Pfizer R&D Clinical Operations / Medical Monitor

2. Study Synopsis & Rationale

2.1 Study Objective **Primary Objective:** To evaluate the safety, tolerability, and early signals of anti-tumor activity of PF-07820435 when administered alone (Part 1A) or in combination with sasanlimab (Part 1B; Part 2) in patients with selected advanced or metastatic solid tumors. **Secondary Objectives:** To evaluate the pharmacokinetic (PK) and pharmacodynamic (PD) profiles of PF-07820435, and to establish the Recommended Phase 2 Dose (RP2D) for combination expansion.

2.2 Background & Rationale To evaluate PF-07820435, an orally available agonist of the STING (stimulator of interferon genes) protein. STING plays a central role in the body's anti-cancer immune response by activating type I interferons and inflammatory pathways. The study assesses whether targeting pattern recognition receptors (PRRs) can overcome resistance to existing immunotherapies (like anti-PD-1) in advanced solid tumors. *(Note: The trial was terminated in early 2025 due to strategic business reasons, not due to safety or efficacy concerns)*

3. Study Design & Methodology

3.1 Design Framework **Study Type:** Interventional **Control Type:** Single-arm (Dose Escalation and Expansion) **Blinding:** Open-label **Randomization:** N/A (Dose escalation cohorts)

3.2 Planned Enrollment & Duration **Target \$N\$:** 9 participants enrolled (Actual enrollment before early termination) **Number of Sites:** Multicenter (Global) **Estimated Study Start:** 08-Feb-2024 (Actual) **Estimated Primary Completion:** 03-Jan-2025 (Actual)

4. Subject Selection (Eligibility Criteria)

4.1 Inclusion Criteria 1. Histological or cytological diagnosis of advanced, unresectable, and/or metastatic or relapsed/refractory solid tumor. 2. **Part 1A:** Participants with solid tumors where anti-PD-(L)1 is an established treatment, who progressed on or following prior anti-PD-(L)1 therapy. **Part 1B:** Participants meeting Part 1A criteria, or with "cold" solid tumors where anti-PD-(L)1 therapy is not established. 3. **Part 2:** NSCLC participants must have received platinum-based chemotherapy and anti-PD-(L)1 (and targeted therapies for actionable mutations). Urothelial Carcinoma participants must have received platinum-based chemotherapy, anti-PD-(L)1 therapy, or enfortumab vedotin. 4. At least 1 measurable lesion based on RECIST 1.1 that has not been previously irradiated.

4.2 Exclusion Criteria 1. Active or history of clinically significant gastrointestinal disease or conditions that pose a risk to study treatment absorption. 2. Active or history of pneumonitis/interstitial lung disease, or pulmonary fibrosis requiring systemic steroid therapy. 3. Active or history of clinically significant autoimmune disease requiring chronic systemic immunosuppressive therapy within the recent 2 years. 4. History of severe immune-mediated adverse events or cytokine release syndrome (CRS) related to prior immune modulatory therapy requiring immunosuppressive therapy.

5. Intervention & Treatment Plan

5.1 Investigational Regimen Dosage/Frequency: Ascending dose cohorts of PF-07820435 monotherapy (Part 1A) and in combination with sasanlimab (Part 1B and Part 2). **Route of Administration:** Oral (PF-07820435) and Intravenous (Sasanlimab). **Duration of Treatment:** Continued until disease progression, unacceptable toxicity, withdrawal of consent, or study termination.

5.2 Concomitant & Prohibited Therapy Permitted: Standard supportive care for advanced solid tumors. **Prohibited:** Concurrent chronic systemic immunosuppressive therapy and other investigational therapeutic agents.

6. Study Timeline & Visit Schedule

Visit ID	Window (Days)	Key Activities/Procedures
Screening	Day -28 to 0	Informed Consent, Medical History, Baseline Tumor Imaging (RECIST 1.1), Safety Labs
Baseline	Day 0	Cycle 1 Day 1: First Dose, PK/PD Sampling, Vital Signs
Interim	Cycle Intervals	Safety Labs, DLT Assessment (first 28 days), Efficacy Scans, AEs Tracking
End of Study	Variable	End of Treatment Visit, Final Safety Assessments, 30-Day Follow-Up

7. Outcome Measures (Endpoints)

7.1 Primary Endpoint Definition 1: Number of patients with dose-limiting toxicities (DLTs) in dose escalation (Part 1A and Part 1B). **Measurement Method 1:** Clinical assessment evaluated during the DLT observation period (Baseline through 28 days after first dose). **Definition 2:** Number of patients with adverse events (AEs). **Measurement Method 2:** Characterized by type, frequency, severity (CTCAE v5; CRS by ASTCT), timing, and seriousness up to 2 years.

7.2 Secondary & Exploratory Endpoints * Early signals of anti-tumor activity (Objective Response Rate [ORR] per RECIST 1.1). * Pharmacokinetic (PK) parameters of PF-07820435.

8. Safety & Adverse Event Reporting

Adverse Event (AE) Monitoring: Continuous monitoring of AEs and DLTs graded per NCI CTCAE v5.0 and ASTCT consensus grading for CRS. **Serious Adverse Events (SAEs):** Standard 24-hour reporting timeline to the sponsor and pharmacovigilance teams. **Data Monitoring Committee (DMC):** Internal Safety Review Committee (SRC) responsible for evaluating DLTs before dose escalation to subsequent cohorts.

9. Statistical Analysis Plan (SAP) Summary

Analysis Populations: Safety Analysis Set (all subjects who receive at least one dose of study medication). **Sample Size Justification:** Sample size determined by standard Phase 1 dose-escalation statistical models; actual enrollment halted at N=9\$ due to sponsor's strategic business termination. **Handling of Missing Data:** Participants dropping out prior to completing the DLT evaluation period for reasons other than toxicity are typically replaced.

10. Quality Assurance & Regulatory Compliance

GCP Compliance: This study will be conducted in accordance with Good Clinical Practice (GCP) and the Declaration of Helsinki. **Monitoring Plan:** Regular onsite and remote monitoring of investigator sites by Pfizer Clinical Trial Monitors. **Audit Trail:** Data entered into validated Electronic Data Capture (EDC) systems with full audit trails and data clarification mechanisms.

11. Study Identifiers & Governance

Primary ID: C5391001 **Registry Number:** NCT06285097 **Sponsor/Lead Organization:** Pfizer **Study Title:** A Study of PF-07820435 as a Single Agent and in Combination in Participants With Advanced Solid Tumors **Official Regulatory Title:** A PHASE 1 DOSE ESCALATION AND EXPANSION STUDY TO EVALUATE SAFETY, TOLERABILITY, PHARMACOKINETIC, PHARMACODYNAMIC, AND ANTITUMOR ACTIVITY OF PF-07820435 AS MONOTHERAPY AND IN COMBINATION IN PARTICIPANTS WITH ADVANCED SOLID TUMORS

12. Clinical Framework (Solid Tumors Specific)

Primary Condition: Advanced Solid Tumors (including NSCLC, Urothelial Carcinoma, Melanoma, Colorectal Carcinoma, etc.) **Diagnosis Threshold:** Histological or cytological confirmation of relapsed/refractory or advanced/metastatic disease. **Medical Methodology:** Immuno-oncology; oral STING agonist administration +/- PD-1 pathway blockade. **Optimization Target:** Identifying the MTD (Maximum Tolerated Dose) and Recommended Phase 2 Dose (RP2D).

13. Study Procedures & Methodology

Management Pathway: Intensive Phase 1 dose escalation followed by disease-specific cohort expansion. **Education Protocol (HCP):** Site initiation visits covering the complex PK/PD sampling schedules and CRS management protocols. **Education Protocol (Patient):** Strict adherence instructions for the novel oral therapeutic agent (PF-07820435). **Quality Audit Mechanism:** Real-time medical review of all DLTs and dose-modification decisions.

14. Observation & Follow-Up Schedule

Total Duration: Up to 2 years of active monitoring. **Onsite Visit Cadence:** Weekly or bi-weekly during early cycles (Cycle 1 & 2) for PK/PD and DLT monitoring. **Remote Monitoring Frequency:** Routine query resolution and safety reviews. **Checklist Review Interval:** Prior to the start of every new treatment cycle.

15. Sign-off & Approval

Role	Name	Signature	Date	:--	:--	:--	:--
Principal Investigator	[Name]	_____	[DD-MMM-YYYY]				
Clinical Operations Lead	[Name]	_____	[DD-MMM-YYYY]			Lead	
Statistician	[Name]	_____	[DD-MMM-YYYY]				